

Women's Health Clinical Reference

Provider Source of Truth

Menopause & Hormone Therapy · KORB Health Group · Internal Use Only

This is the single source of truth for the KORB Women's Health program. It supersedes the Menopause / Women's Health document dated 11/26/2025. It supports provider decision-making and does not replace clinical judgment. Sexual health, hair loss, and skin care add-ons live in the Add-On Clinical Reference, not here.

Stages of Menopause

Stage	What defines it
Perimenopause	Hormone fluctuation and irregular periods
Menopause	Twelve months without a period, with significant hormone decline
Postmenopause	Ongoing low hormone levels, with rising osteoporosis and cardiovascular risk

Identifying the Appropriate Candidate

Candidates are symptomatic women with moderate to severe symptoms affecting quality of life:

- Hot flashes and night sweats
- Vaginal dryness or painful intercourse
- Mood disturbance and sleep problems
- Cognitive complaints, often described as brain fog

Early menopause. These women carry higher risk of osteoporosis and cardiovascular disease and generally benefit from hormone therapy until at least the average age of natural menopause, around 51, unless contraindicated.

The window of opportunity. Outcomes are best when therapy begins under age 60 and within ten years of the last menstrual period. Starting inside that window carries lower cardiovascular risk than starting outside it. Note where a patient sits relative to that window before initiating.

Contraindications and When to Refer

Do not prescribe hormone therapy with any of the following. Refer to PCP or OB-GYN.

- History of breast cancer, endometrial cancer, or other estrogen-sensitive cancer

History of venous thromboembolism or a clotting disorder
History of stroke, myocardial infarction, or uncontrolled hypertension
Active liver disease
Unexplained vaginal bleeding
Known or suspected pregnancy
Migraine with aura (estrogen raises ischemic stroke risk in this group)
Active gallbladder disease (relative; oral routes carry more risk than transdermal)

Pharmacy Routing

Route to	When	What they supply
Premier	First choice. The 38 jurisdictions Premier is licensed in.	Estradiol patch and cream, progesterone (commercial or compounded), testosterone cream, combinations
Belmar	The 13 jurisdictions Premier does not cover: AK, AL, AR, CA, HI, IA, ID, IN, MA, MN, NH, SC, WA	Estradiol cream, progesterone capsules, testosterone cream, combinations
Local pharmacy	Only when the patient wants to use insurance	Commercially available products only: estradiol patches and progesterone capsules

Never send a topical cream or any testosterone to a local pharmacy. Those are compounded and are not commercially available. If a patient elects the local pharmacy route to use insurance, they can receive an estradiol patch and progesterone capsules only. Anything else stays with Premier or Belmar.

Belmar covers all 50 states and DC, which is why they are the second source. Using them for the non-Premier states avoids having to source from a third pharmacy. FarmaKeio is no longer used in Women's Health; they were the original pharmacy and were dropped when they stopped shipping to California.

Compounded products require a Medical Necessity Statement. Belmar raised this specifically for estradiol cream, but it applies to any compounded preparation. Include it when the prescription goes to Premier or Belmar for a compounded item.

Cream Mechanics

One click is 0.25 mL. A 30 mL dispenser therefore holds 120 clicks.

Percent converts to mg per mL by multiplying by 10. A 2% cream is 20 mg/mL, so one click delivers 5 mg.

A 0.6% estrogen cream is 6 mg/mL, so one click delivers 1.5 mg.

Write prescriptions in both mg and clicks. The pharmacies want the directions stated that way.

Apply to hairless, thin skin: inner thigh, labia, back of the upper arm, or forearm.
Let it dry fully before skin contact with another person, and wash hands after applying.
Patches go on the lower abdomen or upper buttock.

Estradiol

First-line for patients WITHOUT a uterus. Transdermal delivery gives steady levels and carries lower clot risk than oral.

Unopposed estrogen in a woman with a uterus causes endometrial hyperplasia and can lead to cancer. If she has a uterus, progesterone is not optional.

Form	Pharmacy	Strengths available
Patch	Premier or local pharmacy	0.025, 0.0375, 0.05, 0.075, and 0.1 mg per day
Cream	Belmar	0.2% (1 click = 0.5 mg), 0.4% (1 mg), 0.8% (2 mg) — 30 mL
Cream	Premier	0.2% (1 click = 0.5 mg), 0.6% (1 click = 1.5 mg) — 30 mL

Cream strengths are not the same at both pharmacies. Belmar offers 0.2, 0.4, and 0.8 percent. Premier offers 0.2 and 0.6. Only 0.2% exists at both. If you move a patient between pharmacies you must re-select the strength and recalculate the dose in mg. Do not carry the percentage across assuming it exists.

Progesterone

Required alongside estrogen in any woman with a uterus, to protect against endometrial cancer. Also helps with hot flashes, night sweats, and mood. Taken at night, where it supports sleep.

Start at 200 mg nightly. Increase to 300 mg if needed.

Regular cycles: cycle 21 days on, 7 days off, alongside estrogen cream or patch.

No regular cycles: take daily.

300 mg is only available compounded. A commercial equivalent means one 200 mg plus one 100 mg capsule.

Patient	Schedule	Quantity per 90 days
Still cycling	21 on, 7 off	67 capsules
Postmenopausal	Daily	90 capsules

Do not use progesterone if the patient has a peanut allergy.

Premier supplies both commercial and compounded progesterone. When both are available you must state which one you want on the prescription, or the pharmacy is left guessing.

Testosterone

Transdermal bioidentical testosterone supports libido, energy, mood, muscle strength, and cognition. The dose women need is low enough that it must be compounded; there is no commercial product at this strength.

Begin with one click daily of 2% cream, which is 5 mg per click, from a 30 mL dispenser.

Adjust on response and side effects.

Premier or Belmar only. Never to a local pharmacy.

Testosterone is available in Texas and California only, and only from the providers listed below.

Texas: Don Stevenson only.

California: Larisa and LaTonya only.

Testosterone is a Schedule III controlled substance. Prescribing requires a DEA registration in the state where the patient is located, and only these providers hold one. This applies to women's low-dose compounded cream exactly as it does to men's TRT.

Do not offer testosterone to a patient outside Texas or California. Every other hormone in this program is available nationwide through Premier or Belmar; testosterone is the single exception. If you are not one of the providers named above, route the patient to one who is.

Belmar is not used for women's testosterone. They lacked a commercial product matching Premier's 90-day cadence, and their compounded version combines other ingredients that providers did not want.

Combination Products

Combination	Composition	Pharmacy
Estradiol + Progesterone	Estradiol cream or patch, plus progesterone capsule. First-line WITH a uterus.	Premier or Belmar. Local pharmacy: patch plus capsule only.
Testosterone + Estradiol cream	Testosterone 2% (5 mg/click) with Estradiol 0.4% (1 mg/click). One click daily.	Premier or Belmar
Progesterone / Testosterone / Estrogen	Testosterone and estradiol as cream, progesterone as capsule.	Premier or Belmar

Combination	Composition	Pharmacy
Oral four-hormone capsule	Progesterone 100 mg, Testosterone 4 mg, Estriol 0.45 mg, Estradiol 0.45 mg. One daily, 90-day supply. Bills at the three-ingredient tier. Contains testosterone, so the prescriber restriction above applies.	Premier or Belmar

Three of these four combinations contain testosterone, so they are limited to Texas and California and to the providers named in the Testosterone section. Only Estradiol + Progesterone is available nationwide.

The oral combination capsule raises clot risk relative to transdermal routes. Oral estrogen passes through the liver first, which transdermal does not. Weigh this before choosing the oral route, particularly in a patient with any cardiovascular or clotting history.

Titration

There is no standard lab panel for Women's Health, and no lab-based target range. Titration is based on **symptom relief**. Increase or adjust when symptoms persist, and reduce when side effects appear. Do not titrate to a number, because there is no agreed number to titrate to.

This differs deliberately from Men's Health, which titrates to a total testosterone band, and from Functional Health, which titrates to an IGF-1 band. If and when a standard panel is adopted here, this section changes.

Laboratory Testing

At-home testing through Ayumetrix remains available and is priced into several of the bundles below. It is optional, not required, and is not used to set doses.

Panel	Contents
Women's Basic Hormones (saliva)	Estradiol, testosterone, progesterone, DHEA, cortisol x1
Women's Complete Hormones (saliva and blood spot)	Saliva: estradiol, testosterone, progesterone, DHEA, cortisol x4. Blood spot: TSH, free T3, free T4, TPO

Under review. Quest serum testing is being evaluated as a replacement. Earlier versions of this document carried a claim that saliva is superior to serum for topical hormone dosing. That claim originated with the testing vendor, has not been independently verified, and has been removed rather than carried forward. Do not repeat it to patients.

Pricing — Medications

Tiers count ingredients, not prescriptions. A single compounded cream containing three hormones is a three-medication charge, not one. Count every active hormone across everything the patient is receiving, then select the tier.

Tier	Includes	Price	Code
Insurance	90-day supply to the patient's local pharmacy	\$79	WMNHlthINS
One ingredient	Visit, 90-day supply, shipping	\$220	WMNHlth1
Two ingredients	Visit, 90-day supply, shipping	\$250	WMNHlth2
Three ingredients	Visit, 90-day supply, shipping	\$299	WMNHlth3
Oral four-hormone capsule	Four actives, billed at the three-ingredient tier	\$299	WMNHlth3

Examples. Estradiol patch alone is one. Estradiol cream plus progesterone capsule is two. A testosterone and estradiol combined cream is two, despite being one product. Testosterone, estradiol, and progesterone together is three.

Pricing — Optional Labs

Bundle	Price	Code
Basic Hormones, standalone	\$119	LABHlthBsc
Complete Hormones, standalone	\$249	LABHlthCmp
One ingredient + Basic	\$339	WMNHlth1Bsc
One ingredient + Complete	\$469	WMNHlth1Cmp
Two ingredients + Basic	\$369	WMNHlth2Bsc
Two ingredients + Complete	\$499	WMNHlth2Cmp
Three ingredients + Basic	\$418	WMNHlth3Bsc
Three ingredients + Complete	\$548	WMNHlth3Cmp

These eight codes all depend on Ayumetrix. If at-home testing is retired in favour of Quest, every one of them needs rebuilding, while the four medication tiers above are unaffected. That is why the two are separated here. The likely replacement structure is a baseline panel plus a recurring interval, which codes more simply than bundling labs into medication tiers.

Sexual Health: PT-141

PT-141 has moved out of Women's Health and is now a **sexual health add-on available across all programs**, alongside KORB Rise and KORB Electric. Full detail lives in the Add-On Clinical Reference.

PT-141 does not follow the 90-day cadence. The labelled beyond-use date is 45 days, but the 28-day-after-first-puncture rule governs in practice, so it is dispensed as a 28-day supply. Do not schedule it alongside quarterly refills. The patient contacts KORB when they need more.

Side Effects

Hormone	What to watch for
Estrogen	Headache, nausea, bloating, mood swings, breast tenderness and swelling, changes in vaginal bleeding, raised clot risk, raised breast cancer risk, raised endometrial cancer risk when unopposed
Progesterone	Headache, mood swings, breast tenderness, changes in vaginal bleeding
Testosterone	Acne, facial and body hair growth, voice deepening, weight gain, mood swings

Voice deepening from testosterone can be permanent. Counsel on it before starting rather than after it appears, and reduce the dose promptly if a patient reports it.

Open Items

Quest transition. Serum panel under evaluation. Dr. Rose has ordered Women's Health labs through Quest for comparison. Both the lab section and the eight lab-bundled charge codes change when a decision is made.

Pricing tiers when testosterone is excluded. Patients outside Texas and California cannot receive testosterone, so the three-ingredient tier is only reachable for them through combinations that do not include it. Worth confirming with Nick that the tier structure still works for those patients.

No titration targets exist. If a standard panel is adopted, agree the target ranges at the same time so this program matches how Men's Health and Functional Health are governed.

Medical Necessity Statement should be confirmed as a standing requirement for all compounded items rather than only estradiol cream at Belmar.